Burns — Adults: Parkland Fluids, TBSA & Referral

adult burns · casualty / ward

4 mL × kg × %TBSA

PARKLAND (NACL 0.9% IV) IF >10% TBSA

24-h volume (mL) ✚: `4 × weight (kg) × %TBSA`

First 8 h ✚: `½ of total (from time of burn)`

Next 16 h ✚: `remaining ½`

Target urine output ✚: `0.5 mL/kg/h; if inadequate ↑ rate by 50% next hour`

WORKED EXAMPLE — ML/H FIRST 8 H | NEXT 16 H

kg \ %TBSA	15%	20%	30%	40%	50%
50	188 | 94	250 | 125	375 | 188	500 | 250	625 | 312
60	225 | 112	300 | 150	450 | 225	600 | 300	750 | 375
70	262 | 131	350 | 175	525 | 262	700 | 350	875 | 438
80	300 | 150	400 | 200	600 | 300	800 | 400	1000 | 500
90	338 | 169	450 | 225	675 | 338	900 | 450	1125 | 562

RULE OF NINES (ADULT)

Area	%	Area	%
Head & neck	9	Each arm	9
Front of trunk	18	Each leg	18
Back of trunk	18	Perineum	1
Patient's palm	≈1		

SA STG — FIRST AID & TREATMENT

- Cool burns <3 h old with cold tap water ≥30 min; chemical burns: running water ≥30 min ✚
- Electrical burns: cardiac monitor; suspect compartment syndrome ✚
- ≤10% TBSA: oral rehydration solution; >10%: NaCl 0.9% IV ✚
- Morphine IV (total max 10 mg) + paracetamol 0.5–1 g 4–6-hourly; tetanus toxoid 0.5 mL IM if not immunised in 5 y ✚
- Pantoprazole 40 mg IV daily; stop once tolerating enteral feeds ✚

DISCUSS WITH BURNS SPECIALIST

- >15% TBSA, or >10% if age >50 years ✚
- Face, hands, feet, genitalia, perineum, major joints; circumferential burns ✚
- Electrical / lightning, chemical, inhalation injury; major trauma ✚